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To the Hipp Engineering Team,

Most healthcare software is built for administrators, not for clinicians trying to do their jobs. The result is platforms that create paperwork instead of eliminating it. Hipp is attacking the right layer — automating the billing and clinical workflows that cost providers hours every week — and doing it with AI from the ground up. That is the kind of problem I want to work on early in my career.

- › **Healthcare ML in Production:** At **Mphasis**, I built health claim automation workflows using OCR, NLP, and ML pipelines for a healthcare data team — improving document processing accuracy by 25% across 10,000+ daily medical claims. I understand the stakes of getting healthcare data processing right.
- › **Startup AI Engineering, Shipped:** At **Isomer AI**, I shipped 10+ production AI features — multi-model LLM pipelines (Claude, GPT-4o, Gemini), Temporal.io agentic workflows, full-stack React/Node.js — across 923 commits and 7 production deployments. I know what it means to own features from idea to production at a fast-moving startup.
- › **Agentic Systems at Scale:** **AlgoPulse** is an agentic AI orchestration platform I built on Temporal.io and Kafka — sustaining 2,000 async inference requests per second with sub-200ms SSE delivery. Replacing a polling architecture with event-driven streaming under load taught me how production AI systems actually fail.
- › **Infrastructure That Holds:** **CortexQ** is a Kubernetes-native LLM inference platform I built from scratch — KEDA autoscaler, Kopf CRD operator, circuit breaker, EWMA load balancing, OTel + Prometheus + Grafana observability. Scalable healthcare infrastructure needs exactly this: systems that hold under real load and give you visibility when they don't.

Hipp is building the platform that lets providers focus on patients instead of paperwork. I want to help build it. I would be glad to discuss.

Sincerely,

Aditya Kulkarni